

Mt. Ascutney Hospital and Health Center Transformational Grant

January 15th Report: Should identify strategies, partners, resource needs, timeline, and preliminary details, but may not include fully developed operational plans.

By January 15th, 2026 MAHHC will identify strategies, partners (internal & external), resources, and timeline to establish an Age Friendly Program (Center of Excellence) focusing on geriatric care needs such as: fall risk assessment, pharmaceutical review, mental health, advanced care planning, etc. identified through analyzing the target population's needs.

Strategies;

Our intent is to install a Project Manager for the oversight of this work by March 1, 2026. This individual will be tasked with developing, through collaboration with internal and external stakeholders, the programmatic structures needed to achieve: consistent data collection, organizing interagency communication and data sharing, evaluating resource management and coordinating process improvement for our geriatric population. The initial focus will be to facilitate the identification of the current strengths in interagency relationships, resource utilization, and specify relationships that are less robust and/or resources that are not available to our vulnerable population. The Project Manager will work closely with the Community Health Team to achieve the stated goals.

In order to develop a baseline for this project we are utilizing the Blue Print Community Health Team's data that we collected during the 10 months of January 2025-October 2025. In this time period 1132 individuals interacted with the CHT team (through visits, or phone calls) and approximately 6800 documented interactions are available in this data set. Of the 1132 individuals collected in the 10-month period, 900 or 80%, were over the age of 64 at the time of their visits. This clearly demonstrates that the preponderance of the individuals served by the CHT are older members of our community and meet the criteria of our grant. Additionally, the data demonstrates that 22% of the individuals communicated with or had visits with members of the CHT team 6 times or more in the 10-month period. This leads one to assume that this group of individuals has more complex needs or need ongoing interactions to be successful. Now that we are able to identify the high utilizers of services, we want to understand what resources or agencies were associated with this population. This information will delineate what agencies we are referring to the most and should help us to understand any deficits in resources. In the

current state this will require in depth chart reviews but our goal will be to establish metrics that will be more easily collected for use.

Partnerships:

As part of the early phases of this project we have redefined our internal and external partnerships and will continue to expand as the resources and information expand. Ensuring strong collaboration and building sustainable processes to share resources is key to our success.

Program Participation Opportunities (internal):

- Ambulatory care patients (primary and specialty practices)
- Walk-in care patients (services non-medically homed persons)
- ED patients (services non-medically homed persons)
- Inpatients (services non-medically homed persons)
- Volunteers in Actions

Potential Partnerships (external):

- Vermont Free Clinics
- Adult Day Cares (Springfield and Quechee)
- Aging in Hartland (Vermont)
- Aging in Reading West Windsor (Vermont)
- BAYADA Home Health Care
- Better Life Partners (BLP)
- Cedar Hill Continuing Care Community
- Claremont Soup Kitchen (New Hampshire)
- Community Nurse Connection
- Connecticut Valley Addiction Recovery (CVAR)
- COVER Home Repair
- Dartmouth Health Medical Center (DHMC), Interdisciplinary Cancer Clinic
- Dartmouth Hitchcock Medical Center (DHMC)
- Dartmouth Hitchcock Medical Center - Lebanon Clinic
- Gifford Medical Center
- Greater Falls Community Justice Center
- Healthcare and Rehabilitation Services (HCRS)
- Keep NH Moving & RLS & Associates, Inc.
- LISTEN Community Services
- MAHHC/OHC

- MyHealthyVT
Community Health Improvement at Rutland Regional Medical Center
- NHCC & Aging and Disability Resource Center (formally ServiceLink)
- Ottauquechee Health Foundation (OHF)
- Planned Parenthood of Northern NE
- Senior Solutions, Area Agency on Aging (AAA)
- SHARe – Supporting and Helping Asylees and Refugees
- Southeastern Vermont Community Action (SEVCA)
- Southwestern Community Service (SCS)
- Special Needs Support Center (SNSC)
- Springfield Area Parent Child Center (SAPCC)
- Springfield Regional Development Office
- Springfield Supported Housing Program (SSHP)
- State of Vermont Agency of Human Services - Brattleboro & Springfield Districts
- State of Vermont Department of Economic Services
- State of Vermont Department of Health - Children with Special Health Needs (CSHN)
- State of Vermont Department of Vermont Health Access (DVHA) - Vermont Chronic Care Initiative (CCI)
- State of Vermont Workforce Development Division of the Vermont Department of Labor
- State of Vermont, Department of Health, WRJ Health District
- Support and Services at Home (SASH)
- The Family Place
- The Hub, Woodstock Community Trust
- Thompson Senior Center
- Turning Point Recovery Center of Springfield
- Upper Valley Pediatrics PLLC
- Valley Heath Connections
- Valley Regional Community Health Connection
- Valley Regional Hospital (VRH)
- Visiting Nurse and Hospice for Vermont and New Hampshire (VNH)
- White River Family Practice (WRFP)
- Windham & Windsor Housing Trust
- Women's Information Services - Upper Valley (WISE UV)

Resource Needs:

Beyond filling the role of Project Manager there may be a need for re-allocation of time for an analyst to support identifying best source for our data and collecting that data. Furthermore, we suspect there will be opportunities for educational training as we increase our knowledge and develop our work flows. Additionally, we will assess the analytics tools available for establishing and analyzing the data from our electronic medical record or other sources. We plan to include the standards from the Geriatric Certification process, and our local experts to ensure that we have a comprehensive best practice approach for working with our geriatric population.

Timeline:

By March of 2026 we will have a detailed set of metrics regarding the current utilization of resources to identify our current state and to help identify areas of opportunities. We are going to focus our efforts on closing the gaps in services or resources. We have a target of September 2026 to have an established program that addresses the needs of our Geriatric population, provides an age friendly approach and address the Social Determinants of health.